

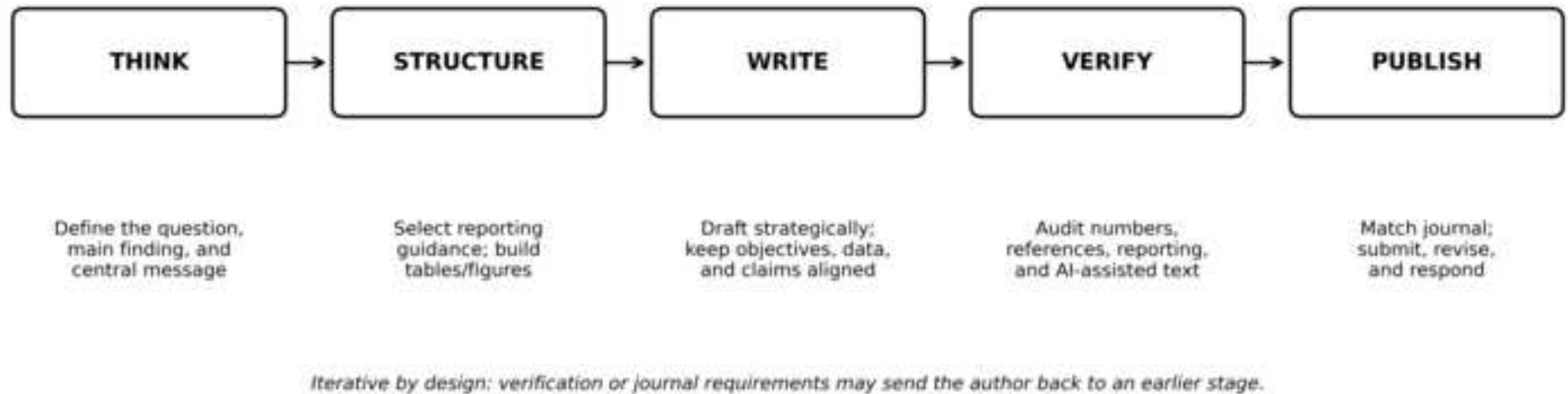
Journal of Graduate Medical Education

Writing Your First Medical Scientific Paper: A Decision-Based Framework for Residents in the Generative AI Era

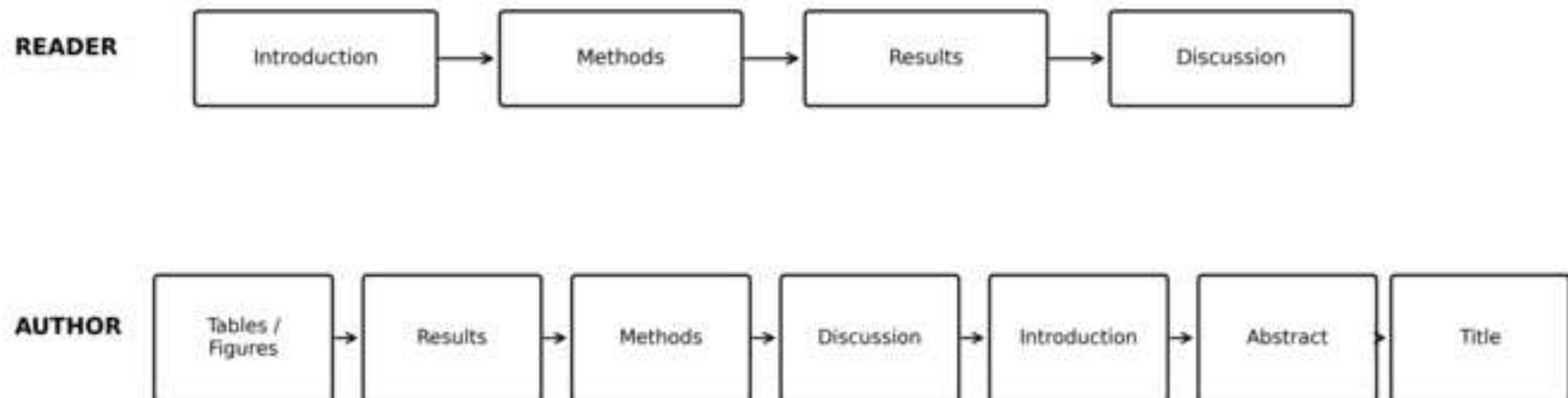
--Manuscript Draft--

Manuscript Number:	JGME-D-26-00893
Full Title:	Writing Your First Medical Scientific Paper: A Decision-Based Framework for Residents in the Generative AI Era
Short Title:	A Decision-Based Framework for Scientific Writing
Article Type:	Reviews
Keywords:	scientific writing; Graduate Medical Education; residents; scholarly publishing; narrative review; generative artificial intelligence
Abstract:	Background: First-time medical authors must do more than learn the Introduction-Methods-Results-Discussion (IMRaD) structure. They must decide what the paper says, how evidence should be organized, what reporting standards apply, how claims will be verified, and how generative artificial intelligence (AI) can be used without delegating scientific responsibility. Objective: To synthesize practical and methodological guidance on manuscript preparation and propose an evidence-informed decision framework for residents and other early-career clinicians preparing an original medical manuscript. Methods: In 2026, we conducted a critical narrative review using PubMed/MEDLINE and targeted searches of authoritative publication resources. We purposively sampled literature on novice scientific writing, scientific storytelling, reporting guidelines, peer review, and generative AI. Review methods were informed by JGME guidance for narrative reviews and the Scale for the Assessment of Narrative Review Articles (SANRA). Sources were appraised according to purpose and authority, then synthesized thematically. Results: Existing guidance is strong on manuscript anatomy, section-level advice, and linear publication roadmaps. A complementary gap is decision support across the writing lifecycle. We propose THINK-STRUCTURE-WRITE-VERIFY-PUBLISH: define the scientific message; select reporting guidance and build the evidence skeleton; draft strategically; audit numbers, references, reporting completeness, and AI-assisted content; and tailor the manuscript for submission and revision. Conclusions: A decision-based framework complements, rather than replaces, existing writing guides. It may provide residents and educators with a shared mental model for producing clearer, more trustworthy, and more submission-ready manuscripts.

A decision-based workflow for the first medical manuscript



Read in IMRaD; draft around the evidence



Suggested teaching sequence, not a universal rule. The aim is to stabilize the evidence before polishing the narrative.

Writing Your First Medical Scientific Paper: A Decision-Based Framework for Residents in the Generative AI Era

Narrative Review

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Target journal: Journal of Graduate Medical Education (JGME) | Article type: Review

Abstract

Background: First-time medical authors must do more than learn the Introduction-Methods-Results-Discussion (IMRaD) structure. They must decide what the paper says, how evidence should be organized, what reporting standards apply, how claims will be verified, and how generative artificial intelligence (AI) can be used without delegating scientific responsibility.

Objective: To synthesize practical and methodological guidance on manuscript preparation and propose an evidence-informed decision framework for residents and other early-career clinicians preparing an original medical manuscript.

Methods: In 2026, we conducted a critical narrative review using PubMed/MEDLINE and targeted searches of authoritative publication resources. We purposively sampled literature on novice scientific writing, scientific storytelling, reporting guidelines, peer review, and generative AI. Review methods were informed by JGME guidance for narrative reviews and the Scale for the Assessment of Narrative Review Articles (SANRA). Sources were appraised according to purpose and authority, then synthesized thematically.

Results: Existing guidance is strong on manuscript anatomy, section-level advice, and linear publication roadmaps. A complementary gap is decision support across the writing lifecycle. We propose THINK-STRUCTURE-WRITE-VERIFY-PUBLISH: define the scientific message; select reporting guidance and build the evidence skeleton; draft strategically; audit numbers, references, reporting completeness, and AI-assisted content; and tailor the manuscript for submission and revision.

Conclusions: A decision-based framework complements, rather than replaces, existing writing guides. It may provide residents and educators with a shared mental model for producing clearer, more trustworthy, and more submission-ready manuscripts.

Keywords: scientific writing; graduate medical education; residents; scholarly publishing; narrative review; generative artificial intelligence

Introduction

Writing a first scientific paper is a transition from doing research to communicating an argument that can survive editorial and peer scrutiny. Beginner-oriented guides have made this transition more approachable by explaining IMRaD, providing section-by-section advice, and converting publication into manageable steps.^{1,2,3} Recent primers continue to address the same educational gap among residents and inexperienced authors, suggesting that scientific writing remains inconsistently taught despite its importance to academic development.^{4,5}

The problem is no longer a lack of instructions. The problem is fragmentation. A novice may know what belongs in a Discussion yet still struggle with the decisions that make the entire manuscript coherent: What is the single defensible message? Which results deserve a table or figure? Which reporting guideline should shape the manuscript? What should be drafted first? How can the author verify that each number and citation is trustworthy? Scientific-storytelling work similarly argues that compliance with IMRaD does not, by itself, create a coherent scientific narrative.⁶

The contemporary publication environment adds two pressures. First, reporting guidelines are intended to improve completeness and transparency and are most useful when incorporated while drafting rather than discovered at the end.⁷ Second, generative AI can assist with language and organization but can also produce incorrect, incomplete, biased, or unattributed content. Updated ICMJE guidance makes human authors responsible for AI-assisted material and requires transparent disclosure of its use.^{8,9}

This review therefore asks a practical question relevant to graduate medical education: What decisions should a first-time medical author make before, during, and after drafting to convert completed research into a trustworthy, submission-ready manuscript? We synthesize existing guidance and propose a five-stage framework—THINK, STRUCTURE, WRITE, VERIFY, and PUBLISH—designed as a teaching and mentoring tool for residents across specialties.

Methods

Review Design and Rationale

We conducted a critical narrative review because the aim was to integrate heterogeneous forms of evidence and guidance—educational articles, methodological papers, editorials, reporting standards, and publication-ethics recommendations—and use that synthesis to generate a practical conceptual framework. JGME guidance notes that narrative reviews are appropriate when a broad literature requires interpretation and critique rather than pooled estimation, provided that authors make their boundaries, search choices, analysis, and perspective explicit.^{10,11}

The review was also informed by SANRA, particularly the domains of importance, explicit aims, description of the literature search, appropriate referencing, scientific reasoning, and presentation of relevant endpoint data.¹²

Scope, Search, and Source Selection

The review focused on residents, fellows, medical students approaching postgraduate training, and early-career clinicians preparing original biomedical manuscripts. PubMed/MEDLINE was searched iteratively through August 22, 2026. Search concepts included combinations of “scientific writing,” “manuscript writing,” “first manuscript,” “novice author,” “resident,” “early-career researcher,” “scientific storytelling,” “IMRaD,” “reporting guideline,” “journal selection,” “peer review,” “revision,” “generative artificial intelligence,” and “large language model.” Targeted searches were also performed in the EQUATOR Network, ICMJE, WAME, and JGME author resources. Reference lists of key papers were reviewed for seminal and complementary sources.

We prioritized sources that (1) directly addressed novice or trainee manuscript writing; (2) provided section-level or workflow guidance; (3) addressed narrative coherence or scientific storytelling; (4) represented authoritative reporting, authorship, or AI policy; or (5) described the submission and peer-review process. We did not restrict by study design because much of the relevant literature consists of educational guidance, editorials, and consensus recommendations. Searching and interpretation were iterative and continued until additional sources no longer changed the framework domains or generated a new decision category.

Appraisal, Analysis, and Reflexivity

Evidence was appraised according to function rather than forced into a single hierarchy. Organizational standards (eg, ICMJE and EQUATOR) were treated as authoritative for editorial and reporting practice; empirical educational studies were used when available; methodological and educational papers supported writing strategies; and expert-opinion articles were used to identify practical approaches but were not presented as comparative evidence of superiority. We mapped recurrent recommendations and tensions, then asked what decision a novice author must make at each point in the writing lifecycle.

The author is a resident physician and medical educator who teaches scientific writing. This perspective shaped the review toward practical problems encountered by trainees, including limited time, variable access to continuous mentorship, and uncertainty about responsible AI use. To reduce overreach, workflow recommendations that are not supported by comparative trials are explicitly labeled as teaching strategies rather than universal rules.

Quality and Sufficiency of the Evidence

The evidence base is heterogeneous. Guidance on reporting standards and AI responsibility is supported by authoritative organizations, whereas evidence comparing specific manuscript-writing

sequences is largely experiential or expert-derived. Educational studies support the feasibility and perceived value of structured writing instruction, including self-guided and AI-assisted mentor-guided approaches.^{17,18} However, there is little evidence that one drafting order improves publication acceptance. Accordingly, the framework is best interpreted as an evidence-informed educational synthesis requiring prospective evaluation, not as a prescriptive guideline.

Results

What Existing Beginner Guides Do Well—and What Remains

Existing guides solve important problems. Ecartot and colleagues explain the construction of a scientific article step by step; Ugarte-Gil and Alarcón provide a practical publication roadmap with relevance to Latin American junior researchers; Lawton and colleagues simplify first-manuscript writing into 10 steps; and recent resident-focused primers reinforce section-level writing principles.^{1,2,3,4} A 2026 roadmap extends guidance through journal selection, submission, peer review, rejection, production, dissemination, and ethical AI use.¹³

The remaining opportunity is therefore not another longer list of steps. Across these sources, the dominant organizing unit is the manuscript section, a linear sequence, or the publication stage. We identified a complementary need for a compact decision architecture that links scientific message, reporting structure, drafting, verification, and editorial action. Figure 1 summarizes the proposed framework.

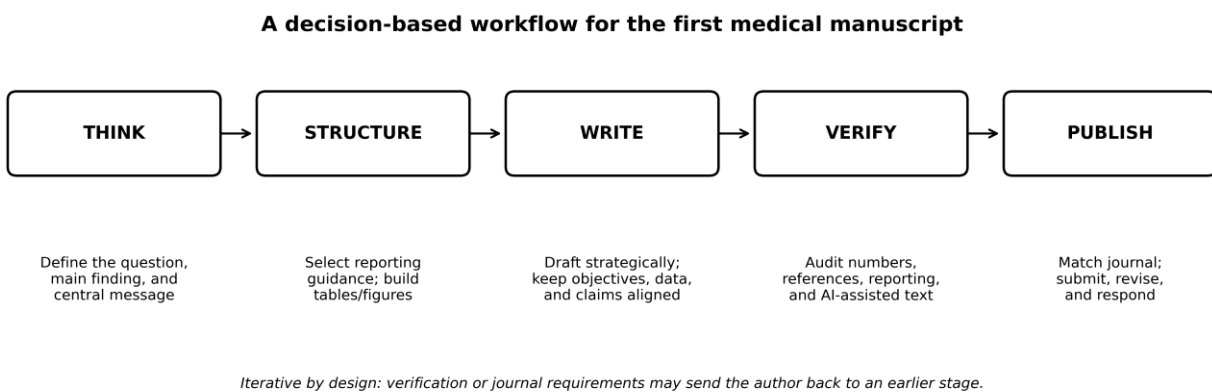


Figure 1. THINK-STRUCTURE-WRITE-VERIFY-PUBLISH: a decision-based workflow for first-time medical authors.

THINK: Define the Scientific Message Before Writing

The first task is conceptual compression: state in one sentence what question was asked, what the principal finding was, and why that finding matters. Scientific-storytelling guidance recommends

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2
3 identifying the resolution of the scientific problem and building the narrative backward from the
4 strongest defensible conclusion.⁶

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6 This “central message” is not a slogan and should not exaggerate novelty. It is a constraint: the
7 objective, displays, Results, and Discussion should converge on the same scientific claim. If they do
8 not, the problem is usually conceptual rather than grammatical. A useful mentoring prompt is: “If the
9 reader remembers only one finding from this paper, what should it be—and do the data actually support
10 that sentence?”
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15 **STRUCTURE: Use Reporting Guidance and Build the Evidence Skeleton**

16 Reporting guidelines should be identified before prose is mature. EQUATOR describes reporting
17 guidelines as tools that help authors include important information and improve completeness and
18 transparency.⁷ Depending on study design, examples include CONSORT, STROBE, PRISMA, STARD,
19 CARE, and TRIPOD.
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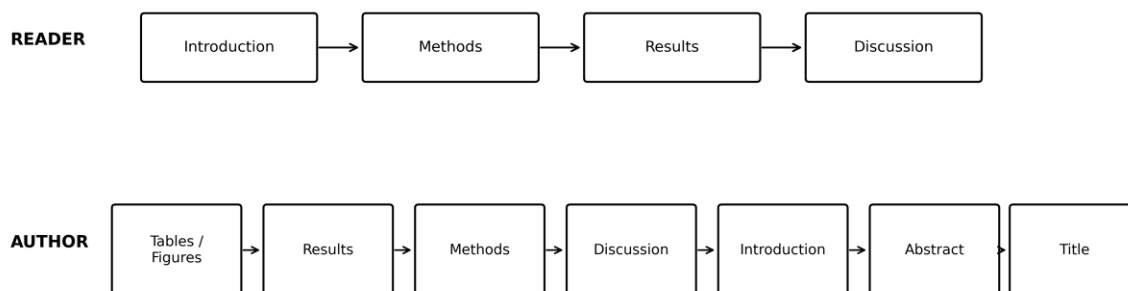
24 We recommend that novice authors then build an “evidence skeleton”: the minimum set of tables and
25 figures needed to answer the study objectives. Each display should have a defined job. If a table does
26 not answer an objective or support the central message, it may be secondary, supplementary, or
27 unnecessary. This step exposes missing analyses and contradictions before they are hidden inside
28 polished prose.
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33 **WRITE: Read in IMRaD; Draft Around the Evidence**

34 IMRaD remains the dominant reading architecture of original biomedical research, but drafting does not
35 need to follow the reading order. Section-level writing guidance emphasizes purposeful construction of
36 the Introduction, Methods, Results, and Discussion, while scientific-storytelling approaches encourage
37 non-linear drafting.^{1,6,14}
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42 For teaching first-time authors, we propose a pragmatic sequence: Tables/Figures → Results →
43 Methods → Discussion → Introduction → Abstract → Title. This is not claimed to be superior to other
44 sequences. Its pedagogic purpose is to stabilize what the study actually found before the author invests
45 heavily in framing and rhetoric.
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Read in IMRaD; draft around the evidence



Suggested teaching sequence, not a universal rule. The aim is to stabilize the evidence before polishing the narrative.

Figure 2. The manuscript is usually read in IMRaD order, but a novice author may benefit from drafting around the evidence first.

Section	Decision question	Common novice failure	Move on when...
Results	What did we find that answers the objectives?	Interpreting findings; narrating every number already shown in a table	Each major result maps to an objective and the displays and text agree
Methods	How exactly was the evidence produced?	Omitting details needed to understand design, variables, analysis, or ethics	A knowledgeable reader can reconstruct what was done
Discussion	What do the findings mean, and how certain are we?	Repeating Results; overclaiming causality; ignoring limitations or alternatives	Interpretation is proportional to design, effect size, uncertainty, and prior evidence
Introduction	What problem and gap make this study necessary?	Writing a textbook review or introducing results prematurely	The final paragraph leads directly to the objective
Abstract/Title	What must a reader understand quickly and accurately?	Adding claims not supported in the manuscript	They faithfully represent the completed paper

Table 1. Decision prompts for drafting the major manuscript sections.

VERIFY: Make Trustworthiness a Separate Work Stage

Verification should be deliberate, not assumed to occur during proofreading. We recommend four audits: (1) numerical consistency across abstract, text, tables, figures, and supplements; (2) analytic consistency between stated methods and reported results; (3) citation fidelity—opening the primary source and confirming that it supports the attached claim; and (4) reporting completeness using the applicable guideline. Verification may send the author back to THINK, STRUCTURE, or WRITE; that is a feature of the framework, not a failure.

Generative AI makes this stage more important. ICMJE states that AI tools cannot be authors, humans remain responsible for submitted material, AI-assisted output should be carefully reviewed, and use of AI should be transparently disclosed. It also warns that AI output may be incorrect, incomplete, biased, or improperly attributed.^{8,9} WAME similarly emphasizes transparency and human accountability.¹⁵

Zone	Examples	Required safeguard
GREEN - Assist	Grammar, readability, alternative wording, organization, redundancy detection	Human author reviews final wording and confirms that scientific meaning is unchanged
YELLOW - Assist + verify	Literature summaries, technical translation, interpretation prompts, statistical explanations, journal-fit suggestions	Check against primary sources, original data, journal policy, and appropriate expertise
RED - Do not delegate	Inventing data, altering results, fabricating citations, deciding authorship, concealing AI use, making final scientific judgments	Human authors retain responsibility and decision authority

Table 2. A risk-based traffic-light model for generative AI during manuscript preparation.

PUBLISH: Design for a Specific Audience and Journal

Submission is not a formatting afterthought. Recent beginner-oriented publication guidance emphasizes manuscript readiness, journal selection, adherence to author instructions, cover letters, peer review, rejection, revision, and resubmission.¹³ JGME itself evaluates manuscripts for relevance to graduate medical education, connection with existing literature and best practices, methodological rigor, and writing quality.¹⁶ Peer-review primers for trainees further support making review and response processes explicit parts of scholarly development.¹⁹

Before submission, authors should confirm journal fit, article type, word and display limits, reporting requirements, authorship and contribution statements, ethics, conflicts of interest, funding, data statements when applicable, and AI disclosure. During revision, reviewer comments should be converted into an action table: concern → response → exact manuscript change → location. When a suggestion is not adopted, the response should explain why rather than simply disagree.

Existing contribution	Strength	Complementary gap addressed here
Step-by-step IMRaD and section guides	Make manuscript anatomy understandable	Our framework links sections to upstream decisions about message, evidence, and verification
Practical roadmaps for junior authors	Demystify the writing/publication sequence	Our framework is explicitly iterative rather than a fixed recipe
Lawton et al 10-step beginner method	Simple, memorable starting process	We add decision prompts, reporting-guideline integration, and a dedicated verification stage
2026 novice-author publication roadmap	Strong coverage of submission through dissemination and ethical AI use	We focus on the full cognitive workflow from scientific message to submission readiness
AI-writing guidance	Defines opportunities and risks	We embed AI governance inside each author decision rather than treating AI as a stand-alone writing tool

Table 3. How the proposed framework complements rather than duplicates prior beginner-oriented guidance.

Box 1. First-Submission Decision Check

- ☐ THINK - I can state the question, principal finding, and central message in one sentence without overstating the data.
- ☐ STRUCTURE - I identified the reporting guideline before finalizing the manuscript and every table/figure has a defined purpose.

- WRITE - Objectives, Methods, Results, Discussion, abstract, and title tell the same scientific story.
- VERIFY - Every key number agrees across manuscript elements; every important citation was opened and checked against the original source.
- VERIFY - Any AI-assisted material was independently checked for accuracy, attribution, confidentiality, and journal-policy compliance.
- PUBLISH - The manuscript is tailored to one journal; authorship, ethics, conflicts, funding, data statements, and AI disclosure are complete.
- PUBLISH - I can respond to reviewer comments by linking each concern to a specific action or reasoned explanation.

Discussion

The main contribution of this review is a change in unit of instruction. Existing literature already teaches sections, steps, and submission logistics well. We propose teaching the first manuscript as a series of decisions that must remain aligned from question to publication. This framing may help residents recognize why a paper feels “stuck”: the problem may be an unstable message (THINK), incomplete reporting or displays (STRUCTURE), section-level execution (WRITE), unverified claims (VERIFY), or journal mismatch (PUBLISH).

The framework also makes explicit a distinction that is increasingly important in the generative-AI era: assistance is not authorship, and fluency is not validity. A polished paragraph can still be scientifically wrong. By making VERIFY a named stage, the model treats factual and citation checking as scholarly work rather than clerical cleanup. This is consistent with contemporary editorial expectations for human accountability and disclosure.

For graduate medical education, the framework is intended to support—not replace—mentorship. Residents often write under clinical time pressure and may have uneven access to experienced research mentors. A shared five-stage vocabulary can make supervision more efficient: instead of “improve the paper,” an educator can identify the decision that requires attention. The model may also be used to structure workshops, longitudinal writing curricula, or peer-writing groups across specialties.

The evidence base warrants caution. Authoritative standards strongly support transparent reporting, human responsibility, and verification, but the optimal order for drafting manuscript sections has not been established experimentally. The proposed writing sequence is therefore a teaching heuristic. Similarly, the framework itself has not yet been prospectively evaluated. A logical next step is to test it in a resident or medical-student writing curriculum using outcomes such as reporting completeness, manuscript quality scored by blinded raters, writing self-efficacy, time to submission, reviewer-requested revisions, and eventual publication.

This review also has methodological limitations. The search was purposive rather than systematic and may have missed relevant literature. Interpretation was shaped by a single author with clinical and teaching experience, which provides practical insight but limits perspective diversity. We attempted to mitigate this by making reflexivity explicit, distinguishing authoritative guidance from expert-derived strategies, and avoiding claims that one drafting sequence is universally superior.

Conclusions

A first-time medical author does not need another description of IMRaD alone. The author needs a map for the decisions that occur before, during, and after IMRaD. THINK-STRUCTURE-WRITE-VERIFY-PUBLISH integrates scientific storytelling, reporting standards, evidence-centered drafting, rigorous verification, journal strategy, and responsible generative-AI use. Its value should ultimately be tested by whether it helps trainees produce clearer, more trustworthy, and more submission-ready manuscripts.

Disclosures

Funding: None.

Conflicts of Interest: The author declares no conflicts of interest.

Ethics Review: Not applicable; this narrative review did not involve human participants or identifiable data.

Generative AI Disclosure: OpenAI ChatGPT was used to assist with literature organization, language refinement, structural drafting, and preparation of original schematic figures. The author reviewed and revised all content, independently verified cited sources and scientific claims, and accepts full responsibility for the manuscript.

References

1. Ecartot F, Seronde MF, Chopard R, Schiele F, Meneveau N. Writing a scientific article: A step-by-step guide for beginners. *Eur Geriatr Med*. 2015;6(6):573-579. doi:10.1016/j.eurger.2015.08.005
2. Ugarte-Gil MF, Alarcón GS. A practical roadmap for writing of medical scientific publications. *J Clin Rheumatol*. 2019;25(7):314-317. doi:10.1097/RHU.0000000000000944
3. Lawton JS, Robich M, Holmes SD. Writing a manuscript for beginners. *Ann Thorac Surg Short Rep*. 2025;3(3):814-817. doi:10.1016/j.atssr.2025.03.013
4. Kristiansen G. How to write a pathology research paper-basic principles and beyond-a primer for residents. *APMIS*. 2025;133(2):e70007. doi:10.1111/apm.70007

5. Weinstein R. Writing manuscripts for peer review. *J Clin Apher.* 2024;39(1):e22108. doi:10.1002/jca.22108
6. Mensh B, Kording K. Finding your scientific story by writing backwards. *Zool Res.* 2023;44(2):371-376. doi:10.24272/j.issn.2095-8137.2022.311
7. Enhancing the QUALity and Transparency Of health Research (EQUATOR) Network. Reporting guidelines. Accessed August 22, 2026.
8. International Committee of Medical Journal Editors. Recommendations for the Conduct, Reporting, Editing, and Publication of Scholarly Work in Medical Journals. Updated January 2026. Accessed August 22, 2026.
9. International Committee of Medical Journal Editors. Use of AI by Authors. Updated January 2026. Accessed August 22, 2026.
10. Sukhera J. Narrative reviews: flexible, rigorous, and practical. *J Grad Med Educ.* 2022;14(4):414-417. doi:10.4300/JGME-D-22-00480.1
11. Sukhera J. Narrative reviews in medical education: key steps for researchers. *J Grad Med Educ.* 2022;14(4):418-419. doi:10.4300/JGME-D-22-00481.1
12. Baethge C, Goldbeck-Wood S, Mertens S. SANRA-a scale for the quality assessment of narrative review articles. *Res Integr Peer Rev.* 2019;4:5. doi:10.1186/s41073-019-0064-8
13. Yalçınkaya B, Çolak AF, Abacıoğlu HB, Özçakar L. Turning manuscripts into publications: a roadmap for novice authors. *Adv Rehabil Sci Pract.* 2026;15:27536351261437943. doi:10.1177/27536351261437943
14. Annesley TM. It was a cold and rainy night: set the scene with a good introduction. *Clin Chem.* 2010;56(5):708-713. doi:10.1373/clinchem.2010.143628
15. World Association of Medical Editors. Chatbots, Generative AI, and Scholarly Manuscripts. Revised May 31, 2023. Accessed August 22, 2026.
16. Journal of Graduate Medical Education. Peer Reviewer Information. Accessed August 22, 2026.
17. Swainson A, Mason MJ, MacMillan FM. An interactive, self-guided tutorial on scientific writing for first year physiology students. *Curr Res Physiol.* 2025;8:100157. doi:10.1016/j.crphys.2025.100157
18. Alrefaie Z, Alhazimi A, Almarabheh A, Madkhali T, Elsamanoudy A. AI assisted, mentor-guided narrative review writing task for medical students, a novel educational strategy to enhance research and academic writing. *Med Teach.* 2026;48(6):1012-1021. doi:10.1080/0142159X.2025.2604240
19. Niznik T, Tavakol SA, Stephens T, Bauer AM, Dunn IF, Graffeo CS. Clinical research primer for medical students: behind the curtain, a framework on peer review for trainees. *J Neurol Surg Rep.* 2025;86(1):e45-e49. doi:10.1055/a-2554-2357

August 24, 2026

Editor-in-Chief
Journal of Graduate Medical Education

Re: “Writing Your First Medical Scientific Paper: A Decision-Based Framework for Residents in the Generative AI Era”

Dear Editor-in-Chief,

Please consider this manuscript for publication as a Review in the Journal of Graduate Medical Education. The review addresses a common problem across specialties: residents are frequently expected to produce scholarly work but receive uneven instruction in how to convert completed research into a coherent, trustworthy, submission-ready manuscript.

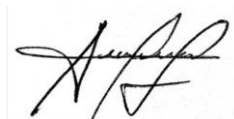
Rather than providing another description of IMRaD or another linear checklist, the manuscript synthesizes existing writing guidance and proposes a five-stage decision framework—THINK, STRUCTURE, WRITE, VERIFY, and PUBLISH. The framework integrates scientific storytelling, reporting guidelines, evidence-centered drafting, verification of data and citations, journal strategy, and responsible use of generative AI. We believe this focus aligns with JGME’s cross-specialty audience and its interest in practical scholarship and graduate medical education.

The manuscript is a critical narrative review. Its methods were informed by JGME guidance for narrative reviews and SANRA, and the manuscript explicitly discusses the quality and limitations of the underlying evidence. The framework is presented as an evidence-informed educational synthesis rather than a prescriptive guideline, and the manuscript identifies prospective educational evaluation as the next step.

This work has not been published elsewhere and is not under consideration by another journal. There was no external funding. The author declares no conflicts of interest. OpenAI ChatGPT was used to assist with literature organization, language refinement, structural drafting, and preparation of original schematic figures; all scientific claims and references were reviewed by the author, who accepts full responsibility for the submitted content. This use is disclosed in the manuscript in accordance with current ICMJE recommendations.

Thank you for considering this manuscript. I believe it can provide residents and GME educators with a concise shared language for diagnosing and teaching the decisions that make scientific writing difficult.

Sincerely,



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Review

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Funding: None.

Conflicts of Interest: The author declares no conflicts of interest.

Ethics Review: Not applicable; this narrative review did not involve human participants or identifiable data.

Generative AI Disclosure: OpenAI ChatGPT was used to assist with literature organization, language refinement, structural drafting, and preparation of original schematic figures. The author reviewed and revised all content, independently verified cited sources and scientific claims, and accepts full responsibility for the manuscript.